

--SUMMARY--

Decision No. 1170/22

16-Sep-2022

C.Zehr

- Preexisting condition (ankle)
- Second Injury and Enhancement Fund {SIEF} (severity of accident)
- Second Injury and Enhancement Fund {SIEF} (severity of preexisting condition)

The issue under appeal was whether the employer was entitled to an increase in the SIEF relief with respect to the costs of the worker's claim.

The appeal was allowed.

The Vice-Chair found that the severity of the accident in this case was moderate and the significance of the worker's pre-existing conditions was major. As a result, the employer was entitled to 75% SIEF relief of the costs of the claim.

The Vice-Chair agreed with the principles in the case law, in that factors such as environmental factors or the mechanics at the time of the accident, distinguish one type of slip and fall accident from another when determining the severity of the slip and fall accident in the context of SIEF appeals. It was noted that this approach aligns with OPM Document No. 14-05-03 which stipulates that when evaluating the severity of an accident, the decision-maker considers multiple components such as the mechanics of the accident, the positioning of the body, and the environment at the time of the accident.

The Vice-Chair found that the accident in this case was distinguishable from the accident in Decision No. 813/05 because there were environmental factors present, which increased the risk of injury. In Decision No. 813/05, the worker was walking across a kitchen floor. In this case, the worker was walking on snowy steps. In addition, when considering the worker's body position and the mechanics during the accident in this case, it was noted that the worker's right foot stepped into a "rut." In the Vice-Chair's view, the worker's right foot would have been likely exposed to greater force (i.e., resistance) with his foot in the rut, than had his foot been on a flat surface. In addition, the worker fell, which likely increased the risk of injury, as opposed to if he had remained upright or caught himself.

Therefore, as similarly concluded in Tribunal decisions, when taking into consideration the mechanics, body position and environment involved in the workplace accident of October 30, 2017, the Vice-Chair found that a disabling injury was probable, and therefore, would have been expected. As such, the severity of the accident was moderate.

Although finding the severity of accident to be moderate, the Vice-Chair found that the worker's pre-existing condition played a significant role in the severity and duration of the worker's compensable injury, and the extent of treatment required. As such, the Vice-Chair agreed that the significance of the worker's pre-existing condition in this case was major.

In the current case, although the worker's entitlement to a right ankle injury was approved shortly after the accident date, the conditions for which the worker was granted entitlement were not clearly defined until after the MRI in March 2018 and further assessments and treatment at the Specialty Clinic. As indicated in the Case Manager's decision dated March 6, 2019, the Board found that the worker's compensable right ankle injury was a rupture of the ATFL, a partial tear of the CFL, an acute strain of right peroneal tendon, sesamoiditis and likely symptomatic activation of the pre-existing degenerative changes. In the same decision, the Case Manager found the remaining conditions of degenerative changes, peroneal tendinosis and bipartite sesamoid to be pre-existing conditions. The assessors suggested that the worker's peroneal tendinosis, sesamoiditis with underlying bipartite sesamoid, and complete ATFL rupture were "possibly longstanding" because the worker had sustained right foot sprains in the past.

The Vice-Chair found that, more likely than not, the ATFL and CFL in the worker's right ankle were structurally compromised prior to the workplace accident of October 30, 2017, and, had these ligaments been unaffected prior to the workplace accident, the worker would have likely not sustained the degree of impairment that he did subsequent to the October 30, 2017 accident. The workplace accident likely exacerbated or progressed that which was already existing in the worker's right ankle.

Therefore, the Vice-Chair found that the majority of the diagnoses, which were found to be compensable in this case, involved the exacerbation of, or a pre-disposition which was caused by, the worker's pre-existing conditions. In this

case, the worker did not fracture his right ankle on October 30, 2017; rather, he experienced a strain, which would have been expected and disabling to a degree based on the nature of the accident. However, this ankle injury had to be casted for several months and the worker had to undergo eight months of physiotherapy treatment. Accordingly, the Vice-Chair found the significance of the worker's pre-existing conditions in this case to be major.

14 Pages

References: Act Citation

- WSIA

Other Case Reference

- [w4222s]
- TRIBUNAL DECISIONS CONSIDERED: Decisions No. 813/05, 2005 ONWSIAT 1211 distd; 1021/12, 2012 ONWSIAT 1907 refd to; 1297/20, 2020 ONWSIAT 1850 refd to; 909/07, 2007 ONWSIAT 1534 refd to; 664/09, 2009 ONWSIAT 910 refd to; 615/08, 2008 ONWSIAT 720 refd to; 449/06, 2006 ONWSIAT 739 refd to; 1238/07, 2007 ONWSIAT 1403 refd to; 1320/15, 2015 ONWSIAT 1522 refd to; 159/20, 2020 ONWSIAT 273 refd to; 1582/07, 2007 ONWSIAT 1751 refd to

Style of Cause:

Neutral Citation: 2022 ONWSIAT 1458



WORKPLACE SAFETY AND INSURANCE APPEALS TRIBUNAL

DECISION NO. 1170/22

BEFORE:

C. Zehr: Vice-Chair

HEARING:

August 15, 2022 at Toronto
Written

DATE OF DECISION:

September 16, 2022

NEUTRAL CITATION:

2022 ONWSIAT 1458

DECISION(S) UNDER APPEAL: WSIB Appeals Resolution Officer (ARO) decision dated
May 10, 2021

APPEARANCES:**For the worker:**

Not participating

For the employer:

K. McBride, Lawyer

Interpreter:

Not applicable

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REASONS

(i) Introduction

[1] The employer is appealing the decision of the ARO, dated May 10, 2021. In the related claim, the worker sustained a right ankle injury on October 30, 2017 while in the course of his employment. The Workplace Safety and Insurance Board (the Board) granted the worker entitlement for a right ankle injury and health care benefits. The Board subsequently found that the worker had a related permanent impairment and granted the worker entitlement for a 1% Non-economic Loss (NEL) award.

[2] The ARO, in his decision dated May 10, 2021, granted the employer entitlement for 25% relief of the claim costs under the Second Injury Enhancement Fund (SIEF).

[3] In this appeal, the employer is requesting SIEF cost relief of 90% to 100%, and the employer representative has provided her submissions in writing to the Tribunal. The worker advised the Tribunal that he would be participating in the appeal; however, as noted in the Tribunal's correspondence dated June 1, 2022, to both workplace parties, the worker confirmed that he would not be providing submissions to the Tribunal in regard to this appeal.

(ii) Issues

[4] The issue under appeal is whether the employer is entitled to an increase in the SIEF relief with respect to the costs of the worker's claim.

(iii) Background

[5] On October 30, 2017, the now 50 year old worker, who had worked for the employer as a courier driver since 1998, injured his right ankle while in the course of his work duties. At the time of the accident, the worker stepped onto a snow-covered step with his right foot and he rolled his right ankle. The worker filed a claim with the Board and the Board granted the worker initial entitlement for a right ankle injury, along with health care benefits.

[6] The worker sought medical attention on the day of the accident, and x-rays of the worker's right foot were conducted, which ruled out a fracture. His right ankle was initially put into an air cast, and later, a regular cast, which was eventually removed on December 28, 2017. The worker began physiotherapy shortly after the cast was removed.

[7] Immediately following the accident, the employer offered the worker modified duties and the worker returned to modified work at no wage loss. The worker continued to perform modified duties as he participated in treatment and he eventually resumed his regular duties on August 13, 2018.

[8] In the meantime, the Board referred the worker to a Foot and Ankle Specialty Clinic, and on February 15, 2018, the assessors diagnosed the worker with an "ankle sprain with concomitant peroneal tendon injury." However, the assessors recommended an MRI, which took place on March 11, 2018. The MRI confirmed the diagnosis of sesamoiditis of the bipartite sesamoid. After further assessments, the Specialty Clinic provided a diagnosis of right peroneal tendinosis, sesamoiditis with underlying bipartite sesamoid, and a complete anterior talo-fibular ligament (ATFL) rupture, which the Specialty Clinic advised was "possibly longstanding," since the worker had sustained right ankle sprains in the past.

- [9] The Board sponsored eight months of physiotherapy at the Specialty Clinic and the worker was discharged from this treatment on August 8, 2018. Upon completion of this treatment, the Case Manager referred the worker's case to a medical consultant, Dr. Mamalias, for review.
- [10] Based on medical evidence from the Specialty Clinic and Dr. Mamalias's memorandum dated July 26, 2018, the Case Manager, in a decision dated March 6, 2019, found that the worker's compensable right ankle injury was a rupture of the ATFL, partial tear of the calcaneofibular ligament (CFL), acute strain of the right peroneal tendon, sesamoiditis and a likely symptomatic activation of the pre-existing degenerative changes. The Case Manager denied the worker entitlement for the remaining conditions of degenerative changes, peroneal tendinosis and bipartite sesamoid, as Dr. Mamalias opined that these conditions were pre-existing and not caused by the accident on October 30, 2017.
- [11] Also, in her March 6, 2019 decision, the Case Manager determined that the worker's compensable condition had reached maximum medical recovery (MMR) as of August 8, 2018. The Case Manager found that, at MMR, the worker had fully recovered from the right sesamoiditis, partial tear of the right CFL, acute strain of the right peroneal tendon and symptomatic activation of the pre-existing degenerative changes. However, the Case Manager found that, at MMR, a permanent impairment remained in relation to the left ATFL rupture.
- [12] On March 7, 2019, based on the Specialty Clinic's Discharge report of August 8, 2018, the Case Manager found that the worker, with his permanent impairment, was able to resume full regular duties, but with caution on uneven surfaces and use of an ankle brace as needed.
- [13] As a result, the worker's case was referred for a Non-economic Loss (NEL) determination. On July 11, 2019, the NEL Clinical Specialist granted the worker a NEL award of 1% for the related permanent impairment in the right ankle.
- [14] In correspondence dated October 3, 2019, the employer requested entitlement for SIEF relief of the costs of the claim. The employer claimed that the worker's recovery from the accepted right ankle diagnosis was prolonged beyond the usual healing time frame. The employer submitted that the worker had a major pre-existing degenerative condition.
- [15] In a decision dated January 20, 2020, the Case Manager found that the severity of the accident would be considered moderate, since the accident would have reasonably been expected to result in a disabling injury. In regard to the significance of the worker's pre-existing condition, the Case Manager found the pre-existing condition to be minor. She indicated that for a pre-existing condition to be viewed as moderate in severity, she would note one or more surgical procedures, some restrictions on employability or lost time from work in the past, as a result of the pre-existing condition. Accordingly, the Case Manager approved 25% SIEF cost relief.
- [16] The employer objected to the quantum of the SIEF relief and the case was referred to the Board's internal Appeals Services Division. In a decision dated May 10, 2021, the ARO concurred with the Case Manager and confirmed the SIEF cost relief of 25%.
- [17] The employer objected to the ARO's decision dated May 10, 2021 and now appeals the decision to this Tribunal.

(iv) Law and policy

[18] Since the worker sustained a work-related injury in October 2017, the *Workplace Safety and Insurance Act, 1997* (the WSIA) is applicable to this appeal. All statutory references in this decision are to the WSIA, as amended, unless otherwise stated. Pursuant to section 126 of the WSIA, the Board advised that the following *Operational Policy Manual* (OPM) Policy Packages, Revision #9, are applicable to the subject matter of this appeal:

- #247 - SIEF
- #299 – Decision Making/Merits and Justice

[19] All of the Board policies within these packages were considered when rendering my decision and relevant excerpts from the policy and WSIA are noted below.

[20] The Board’s authority to establish the Second Injury and Enhancement Fund (SIEF) derives from section 98 of the WSIA, which states:

98 (1) The Board may establish a special reserve fund to meet losses that may arise from a disaster or other circumstance that, in the opinion of the Board, would unfairly burden the employers in any class.

[21] I have considered these policies as necessary in deciding the issues in this appeal, including OPM Document No. 14-05-03, entitled “Second Injury and Enhancement Fund.” This policy provides in part:

Policy

If a prior disability caused or contributed to the compensable accident, or if the period resulting from an accident becomes prolonged or enhanced due to a pre-existing condition, all or part of the compensation and health care costs may be transferred from the accident employer in Schedule 1 to the SIEF.

Both physical and psychological disabilities are included.

Guidelines

There is no provision in the Act for the Fund to apply to Schedule II employers.

In situations where alcoholism plays a role in the causation of an accident, it is not considered to be a pre-existing condition with regard to the application of SIEF relief.

The objectives of this policy are to provide employers with financial relief when a pre-existing condition enhances or prolongs a work-related disability. It thereby encourages employers to hire workers with disabilities.

Definitions

Pre-accident disability is defined as a condition which has produced periods of disability in the past requiring treatment and disrupting employment.

Pre-existing condition is defined as an underlying or asymptomatic condition which only becomes manifest post-accident.

(...)

SIEF-application to employer costs

Medical significance of pre-existing condition*	Severity of accident**	Percentage of cost transfer***
Minor	Minor Moderate Major	50% 25% 0%
Moderate	Minor Moderate Major	75% 50% 25%
Major	Minor Moderate Major	90%-100% 75% 50%

NOTES

* The medical significance of a condition is assessed in terms of the extent that it makes the worker liable to develop a disability of greater severity than a normal person. An associated pre-accident disability may not exist.

With psychological conditions, the possibility of prior psychic trauma resulting from life experience could be considered as evidence of vulnerability, and justify recommending relief to the employer, even in the absence of pre-existing psychological impairment.

** The severity of the accident is evaluated in terms of the accident history and approved definitions.

Accident History Components

- mechanics (lift, push, pull, fall, blow, etc.)
- position (kneeling, standing, sitting, squatting, bending, etc.)
- environment (lighting, temperature, weather conditions, terrain, etc.)

Definition – “Severity of Accident”

- Minor: expected to cause non-disabling or minor disabling injury
- Moderate: expected to cause disabling injury
- Major: expected to cause serious disability probable permanent disability

*** The percentage of the total cost of the claim transferred to the SIEF.

[22] As can be seen from the table of relief found in the Board’s SIEF policy, the amount of SIEF relief granted is dependent on two variables: the severity of the accident and the significance of the pre-existing condition.

(v) Analysis

[23] In her written submissions dated May 30, 2022, the employer representative submits that the severity of the accident should be considered minor and the worker’s pre-existing condition should be classified as having major significance. Accordingly, the employer representative is requesting that the employer be granted 90% to 100% SIEF cost relief.

[24] Based on the preponderance of evidence, I find that the severity of the accident in this case is moderate and the significance of the worker's pre-existing conditions is major. As a result, the employer is entitled to 75% SIEF relief of the costs of the claim.

[25] The reasons for my decision are set out below.

(a) Severity of accident

[26] When considering the severity of the accident, the Board's policy requires that accidents be characterized as either "minor" (expected to cause non-disabling or minor disabling injury), "moderate" (expected to cause disabling injury) or "major" (expected to cause serious disability, probable permanent disability). In addition to these definitions, the policy suggests that the severity of the accident is to be evaluated in terms of the "accident history components," which include "mechanics" (lift, push, pull, fall, blow, etc.), "position" (kneeling, standing, sitting, squatting, bending, etc.) and "environment" (lighting, temperature, weather conditions, terrain, etc.).

[27] In this case, there is no dispute that the workplace accident of October 30, 2017 occurred; however, I note that the accounts of the accident history varied slightly between some of the documents. As a result, I must first confirm the facts of the accident history before assessing whether the severity of that accident is "minor," "moderate" or "major."

[28] I note that, when summarizing the claim in her written submission, the employer representative noted that on October 30, 2017 the worker reported that "he [had walked] up the stairs at a customer location when he slipped on the snow-covered step and rolled his right ankle." I observe that this description of the accident is consistent with the accident history that was recorded on the Employer's Report of Injury (Form 7) dated November 2, 2017. However, later in the body of her written submissions, and when addressing the severity of the accident, the employer representative noted the Case Manager's account of the accident history in the decision dated January 22, 2020. In her decision dated January 22, 2020, the Case Manager stated that "the injury involved a slip from a snow covered step (additional peril) and a twisting of the right ankle in a rut."

[29] In the Case Manager's corresponding memorandum dated January 22, 2020, the Case Manager noted the accident history as follows:

[The worker] stepped off the back bumper of his utility truck and stepped into a rut on a set of stairs with his right foot. [The worker] advised that as he put all of his weight onto his right leg he fell due to the rut; he [could] not recall if he rolled his ankle.

[30] I observe that the initial medical reports in this case, such as the Health Professional's Report (Form 8) dated October 30, 2017 and the first report from the Orthopedic Clinic that was involved in the casting of the worker's foot, dated November 13, 2017, indicate that the worker was stepping out of a truck when he slipped on a step.

[31] I note that the majority of reports pertaining to the accident history indicate that the worker was stepping onto a stair, while coming out of the back of his truck. In my view, the assessment of the severity of the accident does not turn on whether the worker was stepping out of his truck onto a step or on a step at a customer location. There is no dispute that, at the time of the accident, the worker was stepping onto a step with his right foot, and based on the preponderance of evidence, including that from the employer, that step was snow-covered.

[32] I also accept that the worker stepped into “a rut” with his right foot at the time of the accident. I find that the preponderance of evidence appears to include this factor (i.e., the worker stepping into a “rut”) when noting the accident history. For instance, “the rut” was noted in the Form 8 dated October 30, 2017, the physiotherapist’s initial assessment dated January 5, 2018, and the Case Manager’s memorandum dated January 30, 2018, which noted the Case Manager’s direct conversation with the worker on January 30, 2018.

[33] I also accept that the worker either rolled his right ankle at the time of the accident. The rolling of the ankle does not appear to be disputed by the employer noting that this detail was included in the accident history on the employer’s Form 7. I also note that the more contemporaneous medical reports indicate that the worker rolled his ankle at the time of the accident.

[34] Finally I also accept that the worker fell, after he slipped. I observe that most of the contemporaneous reports are silent on the outcome of the slip and whether the worker caught himself or fell. However, I note that during a conversation between the Case Manager and the worker, on January 30, 2018, which was the first time that the Board received an account of the accident history directly from the worker, the worker stated that he had fallen at time of the accident.

[35] Therefore, based on my analysis above, I accept that, on October 30, 2017, while stepping onto a snow-covered step, regardless of whether that step was from his truck or at a customer’s location, the worker’s right foot stepped into a “rut,” causing the worker’s right ankle to roll and the worker to fall. This accident history is generally consistent with what the Board accepted when speaking with the worker on and subsequent to January 30, 2018. I also found that the employer did not provide any substantive evidence to dispute this account of the accident history. As a result, I will consider this accident history when determining the severity of the accident.

[36] The employer representative submits that the severity of the accident on October 30, 2017 should be categorized as “minor.” The employer representative referred me to *Decision No. 813/05*, 2005 ONWSIAT 1211, as supporting this submission.

[37] I note that the Vice-Chair, in *Decision No. 813/05*, found that the worker’s slip and fall, in that case, was a minor accident, since it would have been “expected to cause non-disabling or minor disabling injury.” The Vice-Chair in *Decision No. 813/05* cogently states:

The results of a slip and fall could, in theory, vary from no injuries at all to, in an extreme case, death. Rather than focusing on the results of the fall in determining its severity, I am of the view that Board policy requires one to consider the most likely or expected result of an incident, rather than the actual result. While the results can vary, I am of the view that a slip and fall on a kitchen floor would, more likely than not, be expected to cause a non-disabling or minor disabling injury. Had the worker fallen from a height, or fallen onto another object or into a hole for example, the characterization of the incident may well have been different. In this case however, I am satisfied that the accident severity ought to have been characterized as “minor”....

[38] I agree with the principle noted in *Decision No. 813/05* that the Board policy requires one to consider the most likely or expected result of the accident, rather than the actual result of the accident, when considering the severity of that accident. I recognize that the employer representative emphasized this principle in *Decision 813/05* because, in the case before me, the worker sustained a significant injury. The worker’s ankle had to be casted; he was diagnosed

with multiple compensable conditions in the right foot; he participated in an extended amount of treatment (i.e. eight months of physiotherapy); and he was left with a permanent impairment. Nonetheless, I am in agreement with the employer representative's reference to *Decision No. 813/05* in that the actual resulting injury in this case does not determine the severity of the slip and fall accident, but rather, I must consider what would be the expected of or most likely result from the accident history noted above.

[39] This principle is similar to that noted in *Decision No. 1021/12*, 2012 ONWSIAT 1907 . In *Decision No. 1021/12*, the Vice-Chair confirmed that the actual injuries are not considered when determining the severity of the accident for SIEF purposes, but rather, it is the extent of disability, which the mechanics of the accident would reasonably be expected to cause.

[40] The employer representative also drew my attention to *Decision No. 813/05*, because the Vice-Chair in that decision categorized the worker's slip and fall as a "minor" accident. The employer representative is suggesting that the current worker's accident would have similar characteristics to the accident in *Decision No. 813/05*, since it was also a slip and fall accident.

[41] There are a plethora of Tribunal decisions that address the issue of whether a slip and fall accident is minor or moderate. In *Decision No. 1297/20*, 2020 ONWSIAT 1850, the Vice-Chair noted multiple decisions in which a slip and fall accident was determined to be minor in severity. Those decisions were *Decision Nos. 813/05; 909/07*, 2007 ONWSIAT 1534; *664/09*, 2009 ONWSIAT 910; *615/08*, 2008 ONWSIAT 720; *449/06*, 2006 ONWSIAT 739; *1238/07*, 2007 ONWSIAT 1403; and *1320/15*, 2015 ONWSIAT 1522. However, the Vice-Chair in *Decision No. 1297/20* noted multiple decision in which a slip and fall on ice was determined to be a moderate accident because of the environmental factor involved in the accident. The Vice-Chair in *Decision No. 1297/20* noted the following:

[21] Turning to the various Tribunal decisions provided by the employer's representative, I have determined that I will place little weight on these for various reasons. Firstly, I note that only two of the decisions (*Decisions No. 615/08 and 1320/15*) involved a slip and fall on ice. Secondly, and importantly, I have reviewed Tribunal jurisprudence and I note a significant number of Tribunal decisions (some of which are cited below) have determined that a slip and fall on ice constitutes a moderate accident and not a minor accident. (See *Decision Nos. 966/19; 1495/12; 167/13; 244/13; 2275/12; 961/19; 1807/16; 471/20 and 2215/18*). In particular, I note that the Vice-Chair in *Decision No. 244/13* stated:

I note also that it is not necessary that the accident mechanics be expected to cause a permanent impairment for the accident severity to be moderate. It also is not necessary for the accident to be expected to cause a worker to become totally disabled for any extended period of time. In my view, if it is not unexpected that the accident has caused the worker to lose time from work or that the worker has required modified work in more than a minor respect, then in my view the accident constitutes a disabling injury within this provision.

While Mr. Senicar refers me to *Decision Nos. 813/05 and 1238/07* as examples of findings that slip and fall accidents may be minor accidents, neither of those decisions addressed a fall on ice. In any event, each case must be addressed on its specific facts.

In my view, although it may have been unexpected that this accident would have caused a permanent impairment, which is what the worker suffered, I do not consider it unexpected that he suffered an injury that required time off work and/or a requirement for modified work. This was not an injury resulting from a mere twist or turn or stumble. The worker fell to the ground. Further, because ice is a slippery

service, a fall on ice may involve more jerking or twisting than a fall on a less slippery surface. Ice is a hard surface upon which to land and this fact may also increase the degree of injury. I do not consider it unexpected that some period of disability resulted from a fall of this type. I find that the accident was moderate in severity.

[42] In addition, the Vice-Chair in *Decision No. 159/20*, 2020 ONWSIAT 273, noted other factors, which might affect the categorization of a slip and fall accident when determining SIEF relief, such as whether the worker fell or did not fall at the time of the accident. The Vice-Chair, in *Decision No. 159/20* states:

[30] ...In fact, the more recent line of Tribunal decisions regarding slip and fall accidents in the context of SIEF related appeals, have determined that these types of accidents are of moderate severity. These decisions draw a distinction between a slip, which does not result in a fall to the ground, and a slip and fall type accident where the worker actually falls to the ground. These decisions conclude that the former type of accident, which is a more common or frequent occurrence, can be considered a minor accident expecting to cause a non-disabling or minor disabling injury. With respect to the latter there appears to be a general consensus that a slip and fall injury can be expected to cause a disabling injury, recognizing that these types of injuries may vary significantly, from no injuries at all to death.

[31] For example, *Decision No. 3769/17* concluded a slip and fall accident was of moderate severity. This decision has been referenced, with approval, in several subsequent Tribunal decisions including, *Decision Nos. 684/19, 184/19, 598/19, and 2106/18*.

[43] As can be noted in the case law above, some slip and fall accidents have been categorized as minor, and others as moderate, in past decisions. Indeed, there are likely some cases in which the severity of the accident would be categorized as major, depending on the individual merits of the case (e.g., a slip and fall from a significant height).

[44] I agree with the principles in the case law noted above, in that factors such as environmental factors or the mechanics at the time of the accident distinguish one type of slip and fall accident from another when determining the severity of the slip and fall accident in the context of SIEF appeals. This approach aligns with OPM Document No. 14-05-03 noted above. OPM Document No. 14-05-03 stipulates that, when evaluating the severity of an accident, the decision-maker considers multiple components such as the mechanics of the accident, the positioning of the body and the environment at the time of the accident.

[45] I now turn to the case before me and the accident history noted above. I find that the accident in this case is distinguishable from the accident in *Decision No. 813/05* because there were environmental factors, which increased the risk of injury. In *Decision No. 813/05*, the worker was walking across a kitchen floor. In the case before me, the worker was walking on steps and there was snow on the steps. In addition, when considering the worker's body position and the mechanics during the accident in this case, I note that the worker's right foot stepped into a "rut." In my view, the worker's right foot would have been likely exposed to greater force (i.e., resistance) with his foot in the rut, than had his foot been on a flat surface. In addition, the worker fell, which likely increased the risk of injury, as opposed to if he had remained upright or caught himself.

[46] Therefore, as similarly concluded in Tribunal decisions, as noted above, when taking into consideration the mechanics, body position and environment involved in the workplace accident of October 30, 2017, I find that a disabling injury was probable, and therefore, would have been

expected. As such, for the purposes of determining the employer's SIEF cost relief, I find that the severity of the accident is moderate.

(b) Significance of the pre-existing condition

[47] Although finding the severity of accident to be moderate, I find that the worker's pre-existing condition played a significant role in the severity and duration of the worker's compensable injury and the extent of treatment required. As such, I agree with the employer representative that the significance of the worker's pre-existing condition in this case, is major. I explain my reasons for this finding below.

[48] With respect to the matter of the pre-existing condition, Board policy requires that it be characterized as minor, moderate or major. The policy does not define these terms and indicates only that the medical significance of a condition is assessed "in terms of the extent that it makes the worker liable to develop a disability of greater severity than a normal person." These provisions were interpreted as follows in *Decision No. 1582/07*, 2007 ONWSIAT 1751:

I interpret the policy to mean that the medical significance of a pre-existing condition should be considered to be "minor" if it made the worker slightly more liable to develop a disability of greater severity than a normal person, and that it should be considered "major" if it made the worker extremely liable to develop a disability of greater severity than a normal person. If the extent to which the pre-existing condition made the worker more liable to develop a disability of greater severity than a normal person was more than slight, but less than extreme, the medical significance of the pre-existing condition could be considered moderate.

[49] In the current case, although the worker's entitlement to a right ankle injury was approved shortly after the accident date, I note that the conditions for which the worker was granted entitlement was not clearly defined until after the MRI in March 2018 and further assessments and treatment at the Specialty Clinic.

[50] As indicated in the Case Manager's decision dated March 6, 2019, the Board found that the worker's compensable right ankle injury was a rupture of the ATFL, a partial tear of the CFL, an acute strain of right peroneal tendon, sesamoiditis and likely symptomatic activation of the pre-existing degenerative changes. In the same decision, the Case Manager found the remaining conditions of degenerative changes, peroneal tendinosis and bipartite sesamoid to be pre-existing conditions.

[51] I observe from the Specialty Clinic reports that the assessors suggested that the worker's peroneal tendinosis, sesamoiditis with underlying bipartite sesamoid, and complete ATFL rupture were "possibly longstanding" because the worker had sustained right foot sprains in the past.

[52] I note from the Medical Consultant's (Dr. Mamalias's) July 26, 2018 memorandum that Dr. Mamalias contacted the worker's general practitioner, Dr. L. Griffiths, on July 24, 2018 to obtain further information about the worker's pre-existing right ankle/foot condition. According to Dr. Mamalias's memorandum, during this conversation, Dr. Griffiths advised that the worker had undergone three x-rays of his right foot and ankle between 2012 and November 2017, which were negative for acute trauma such as fractures/dislocation. Dr. Griffiths also advised Dr. Mamalias that the worker had attended the Emergency Department in 2012 with regards to his right ankle, and again in 2014, regarding pain in his right foot. Metatarsalgia was queried at the time. From this evidence, I find that the worker experienced a symptomatic right ankle and

foot condition prior to the workplace accident on October 30, 2017. However, Dr. Griffiths also reported to Dr. Mamalias that, in spite of these issues, the worker had been fully functional with his right ankle, prior to the workplace accident on October 30, 2017.

[53] In addition to the medical history provided by Dr. Griffiths, I note that the case materials contain information regarding two work-related right ankle injuries, which the worker sustained in February 2005 and August 2015. According to the Employer's Reports of Injury (Form 7s), on each of these occasions, the worker twisted his right ankle while in the course of his duties. Under these claims, I observe that the worker resumed his regular duties shortly after these accidents.

[54] I return to the current case. When considering the nature of the worker's pre-existing conditions, other than that noted above, I find that the reports from the Specialty Clinic provide minimal information to assist in my decision, because the focus of these reports is on the worker's progress and treatment. However, I note that the employer representative, in her written submissions, often referred to the Medical Consultant's review of July 26, 2018.

[55] I agree with the employer representative that Dr. Mamalias's file review has evidentiary value because, as can be seen from his memorandum, Dr. Mamalias reviewed all of the medical documents in the case file at the time. I am aware that Dr. Mamalias did not examine the worker; however, for the issue at hand, I find that a review of the medical findings and reports, as a whole, is of more value than an assessment of the worker's condition at one point in time, particularly since it took some time in this case to determine the compensable and non-compensable conditions. I also note that, as part of his review, Dr. Mamalias included the medical history provided by Dr. Griffiths and I observe that Dr. Mamalias provided explicit opinions regarding the worker's pre-existing condition in response to the Case Manager's question on the matter. For these reasons, I have placed weight on Dr. Mamalias's opinion and I note that opinion below.

[56] In the referral memorandum to Dr. Mamalias, I note that the Case Manager asked the Medical Consultant to define "which findings can be attributed to a pre-existing condition or natural aging/degeneration?" Dr. Mamalias responded to that question as follows:

It is relevant to note that this worker's medical history is remarkable for prior right ankle injury, and while there is no diagnostic imaging prior to the reported mechanism for this claim that evaluates the soft tissue structures surrounding the right ankle, i.e. ultrasound or MRI, this worker's reports of pre-existing instability in the right ankle possibly suggests a structural compromise to the ligamentous structures of the right ankle from before the work-related injury on October 30, 2017. That being said the worker would appear to have been fully functional with respect to the right ankle leading up to the work-related injury in question, and this was confirmed on discussion with the worker's Family Physician during the completion of this current File Review. The reported mechanism for this claim, involving a rolling of the ankle, would be considered consistent with a mechanism that could plausibly result in acute injury to the lateral supporting ligamentous structures of the right ankle, such as the ATFL and CFL, which are commonly injured during acute ankle sprain type injuries, and this worker presented subsequent to this mechanism with acute pain in this region as well as reports of instability in the ankle, resulting in considerable functional impairment and requiring a period of non-weightbearing/partial weightbearing and use of a cast. Therefore, based on the information available for consideration at the time of this review, what is plausible and likely is that the worker, at the very least, sustained an acute structural exacerbation and likely further progression of already compromised/pre-existing ATFL and CFL ligamentous damage in the form of further rupture/tearing of these ligaments as a direct

result of the work-related injury in question, and at the most sustained completely new structural injuries to these ligaments. Although definitively knowing the extent of this worker's prior ATFL and CFL injuries is not possible given the absence of pre-injury diagnostic investigations evaluating these structures, regardless of the scenario, rather it be a structural exacerbation and progression or completely new structural injury, it is likely that the work-related injury in question has contributed to/resulted in the structural injuries of the ATFL and CFL visualized on the post-injury MRI, and therefore the findings of rupture of the ATFL and partial tear of the CFL would be considered compatible with the work-related injury in question.

With respect to the findings of peroneal tendinosis, this implies a pathology of chronic degeneration to the tendon that is difficult to directly causally relate to the acute/isolated work-related injury in question, and would be in keeping with this worker's pre-existing history of prior ankle injury; and therefore, is likely to be preexisting in nature. That being said, the reported mechanism for this claim of the worker's rolling his ankle, in addition to resulting in acute injury to the lateral ligamentous structures of the ankle, can also cause acute injury to the tendons that cross and support the lateral ankle, including the peroneal tendons, which are commonly injured during acute ankle sprain injuries; and this worker's clinical presentation post-injury of tenderness directly over the peroneal tendon would be in keeping with this injury. Therefore, while there is no evidence of acute structural injury to this tendon, it is this Physician's opinion that what is plausible and likely is that the worker sustained an acute strain of the right peroneal tendon and likely symptomatic activation of pre-existing degenerative changes in this structure as a direct result of the work-related injury in question.

With respect to the findings of bipartite sesamoid, this is a common anatomical variant of the sesamoid bones in the big toe, which is not traumatic in origin, and therefore is likely to be pre-existing in nature. Sesamoiditis essentially describes irritation of the sesamoid bone resulting in pain over the plantar aspect of the toe/foot, which could potentially occur in the setting of an acute trauma to this region. The reported mechanism for this claim describes the worker stepping down from stairs and into a rut, which is plausible to have placed some compressive forces and strain through the plantar aspect of the foot. While it is appreciated that the worker does have a history of pain over the plantar aspect of his feet in the past, i.e. an ER visit noted on discussion with the worker's Family Physician for metatarsalgia in the right foot in 2014, he does not appear to have had any functional limitations in this regard leading up to the work-related injury in question. Subsequent to the reported mechanism for this claim, the worker reported pain over the plantar aspect of his foot, and difficulty with prolonged walking as a result of this. As such, it is this Physician's opinion that the diagnosis of sesamoiditis is likely to be compatible with the work-related injury in question.

[57] As noted above, Dr. Mamalias noted that, as a result of the workplace accident, the worker experienced "considerable functional limitations." However, I note that Dr. Mamalias opined that, at the time of the accident, the worker likely sustained an acute structural exacerbation and likely further progression of the rupture or tear that had already existed in his right ATFL and CFL. I note that Dr. Mamalias opined that the worker's right ATFL and CFL were compromised prior to the workplace accident as a result of the prior right ankle injuries. I note that there is substantive evidence supporting that the worker sustained multiple right ankle sprains prior to the workplace accident of October 30, 2017, which, according to Dr. Mamalias, commonly causes damage to the ATFL and CFL. I also note that the assessors at the Specialty Clinic suggested that some of the worker's conditions were "long-standing" and likely due to the prior right ankle injuries.

[58] I am aware that the worker returned to and sustained his regular duties after the prior right ankle strains. Dr. Griffiths confirmed that the worker's right ankle was "fully functional"

in his job at the time of the workplace accident of October 30, 2017. However, I also note that the worker has been able to sustain his regular duties with the permanent impairment related to the current claim. Therefore, I find that the worker is able to sustain his regular duties with a compromised right ankle condition. As such, the worker's ability to perform his regular duties prior to the workplace accident of October 30, 2017 does not preclude the possibility that his right ankle was compromised prior to the workplace accident.

[59] Based on Dr. Mamalias's review, I find that, more likely than not, the ATFL and CFL in the worker's right ankle were structurally compromised prior to the workplace accident of October 30, 2017, and, had these ligaments been unaffected prior the workplace accident, the worker would have likely not sustained the degree of impairment that he did subsequent to the October 30, 2017 accident. Again, as indicated by Dr. Mamalias, the workplace accident likely exacerbated or progressed that which was already existing in the worker's right ankle.

[60] I also note that the worker had other pre-existing conditions that likely enhanced the worker's impairment and prolonged the treatment required. Dr. Mamalias opined that the worker's peroneal tendinosis in the right ankle was chronic and activated by the workplace injury. The worker was also diagnoses with sesamoiditis after the workplace accident and I note that Dr. Mamalias indicated that the worker had an anatomical variant in the sesamoid bone of the big toe, which was a pre-existing, non-traumatic condition.

[61] Therefore, as supported by my analysis above, I find that the majority of the diagnoses, which were found to be compensable in this case, involved the exacerbation of, or a pre-disposition which was caused by, the worker's pre-existing conditions. In this case, the worker did not fracture his right ankle on October 30, 2017; rather, he experienced a strain, which would have been expected and disabling to a degree based on the nature of the accident. However, this ankle injury had to be casted for several months and the worker had to undergo eight months of physiotherapy treatment. I also note that the Board approved other health care expenses for the worker such as orthotics, custom-made shoes and ankle stockings, and as noted above, this injury resulted in a permanent impairment. Although I find that a disabling injury would have been expected based on the severity of the accident in this case, I also find that the worker's multiple pre-existing conditions made him extremely liable to develop a post-accident condition of greater severity than had he not had the pre-existing conditions. I also find that the worker's pre-existing conditions likely prolonged the treatment, which the worker required.

[62] Accordingly, I find that the significance of the worker's pre-existing conditions in this case, is considered major. Since I find the severity of accident to be moderate and the significance of the pre-existing conditions to be major, in accordance with OPM Document No. 14-05-03, the employer is entitled to 75% SIEF relief of the costs in this claim.

DISPOSITION

[63] The appeal is allowed.

[64] The employer is entitled to 75% cost relief under the SIEF in relation to the worker's claim for the workplace accident on October 30, 2017.

DATED: September 16, 2022

SIGNED: C. Zehr